

Golden State Rehab – Internal Linking, Topical Authority & E-E-A-T Audit

Date: August 10, 2026 **Scope:** 48 indexable English pages (106 HTML files total incl. Spanish mirror and helpers) **Method:** Full-site crawl of the static HTML source. In-body links analyzed separately from sitewide nav/footer boilerplate (~140 boilerplate links per page excluded). Word counts are body-only (hero through end of content). Duplication measured by 6-gram shingle overlap.

Executive Summary

Dimension	Grade	One-line verdict
Internal Linking	B-	Real hubs, descriptive anchors, zero broken links – but the homepage passes no contextual equity to any money page, and the blog is a one-way spoke.
Topical Authority	C+	The condition and level-of-care axes are genuinely well built (~55% of the vertical), but the three axes competitors monetize – insurance-by-carrier, cost, and audience – have zero pages.
E-E-A-T	C+	Strong medical-review infrastructure and real staff with lived experience, undermined by an unsourced outcome claim in the H1, no license numbers, unverifiable reviews, and no accreditation beyond the DHCS license.

The five findings that matter most:

- "100+ Recoveries" in the homepage H1** is an unsourced, undefined outcome claim that contradicts `our-story.html` (which frames the same number as "Clients served"). For a YMYL addiction-treatment site this is the single highest regulatory and trust risk on the site.
- The homepage links to nothing.** Its 31 in-body anchors go to the team page (13×), verify-insurance (7×), and three utility pages. Zero contextual links to any treatment, program, location, or blog page – the site's strongest page passes no equity into any page that ranks.
- The insurance/cost/audience content axes don't exist.** Carrier logos appear on 46 pages with zero carrier content behind them. No `/insurance` hub, no cost page, no detox page, no heroin/benzo pages, no who-we-treat silo. The buildout plan calls these Phase 1-3; almost none of it is built.
- Social proof is unverifiable.** The Google review wall links to image files, not the Google Business Profile; "5.0 on Google" carries no review count; there is zero Review/AggregateRating schema sitewide; staff license numbers are withheld ("available on request") despite being public record.
- The blog can't support the site topically.** Service pages link to blog posts exactly 5 times sitewide – 4 of them the same recycled sentence pointing at the same post. Four of six posts have 1-4 contextual inbound links; five of six have no medical reviewer.

1. Internal Linking – B-

1.1 What's working

- 918 in-body internal anchors** across 60 English pages; median ≈ 12 contextual links per page, mean ≈ 15.
- Anchor text is strong:** only 19 of 918 anchors (2.1%) are generic ("Get Started," "Learn More," "View all"). The rest are descriptive and keyword-rich ("guide to outpatient drug rehab in Los Angeles," "whether insurance covers rehab in California").
- Hubs are real hubs.** `treatments/index.html` (32 out-links, chip nav + unique 15-25-word card descriptions for all 15 children), `programs/index.html` (21), `locations.html` (23, unique city descriptions), `mental-health.html` (27, functions as a secondary hub). `faq.html` is the densest contextual linker on the site (50 contextual out-links).
- Zero broken internal links** across all 60 pages (nav, footer, and body verified; extensionless URLs resolve via `_redirects`).
- Treatment → treatment cross-linking is good** (98 links): every treatment page carries a 3-card related module plus prose links (opioid → fentanyl, cocaine → meth, etc.).
- Blog → service linking is dense:** 29 in-body links from posts into programs/treatments with descriptive anchors.
- hreflang** present on 57 of 61 English pages with a working EN↔ES cross-link structure.

1.2 Orphans and near-orphans

Page	In-body inbound	Notes
amenity-map.html	0	Only reachable via iframe embeds – invisible to the link graph
intake-success.html	0	Form thank-you (acceptable)
terms-and-conditions.html	0	Footer-only (acceptable)
programs/outpatient-rehab.html	1	Worst-supported valuable page on the site – absent from nav, footer, hreflang, and the ES mirror, despite 1,138 body words targeting the head term
blog/does-medi-cal-cover-rehab-in-california.html	1	Blog index only
faq.html, our-story.html, our-facility.html	1 each	Sole source: about.html
treatments/sex-addiction.html, treatments/prescription-drugs.html	1 source each	Treatments index only

1.3 The homepage problem

index.html has **zero in-body links to any treatment, program, location, or blog page**. Its 31 body anchors resolve to: team ×13 (staff carousel), verify-insurance ×7, about, spanish-speaking-treatment, espanol, and tel: links. The "why us" and facility card grids (index.html:234–252, 283–290) are plain <div> s with no anchors. The site's highest-authority page passes no contextual equity into any page meant to rank.

1.4 The blog is a one-way spoke

- Blog → service: 29 links, well-anchored.
- **Service → blog: 5 links sitewide, 4 of which are the same recycled insurance-parity sentence pointing at the same post** (does-insurance-cover-rehab-in-california).
- Zero inbound from services to: cost-of-rehab-in-los-angeles, first-week-of-outpatient-rehab, cbt-vs-dbt-which-is-right, terrified-to-ask-for-help, does-medi-cal-cover-rehab.
- Obvious misses: programs/iop.html never links to the "first week of outpatient" post; treatments/cbt.html / dbt.html never link to the CBT-vs-DBT post; verify-insurance.html (149 inbound links – the site's biggest link sink) has **zero in-body out-links**, including to the three insurance/cost posts.

1.5 Cross-silo link matrix (in-body only)

From ↓ To →	blog	locations	programs	treatments	root pages
blog	34	0	18	11	24
locations	0	33	83	6	104
programs	2	0	98	5	80
treatments	2	0	53	98	79
root pages	1	11	30	32	108

Structural holes: treatments → locations is zero; locations → treatments is 6 links total across 11 city pages (8 city pages link to no treatment at all); programs are a closed silo (98 internal, 5 out to treatments).

1.6 Smaller issues

- Link equity is lopsided: dual-diagnosis receives 29 in-body links from 23 sources; sex-addiction and prescription-drugs get 2 each from 1 source.
- treatments/cbt.html:180,186 – "OCD & Intrusive Thoughts" and "Insomnia & Sleep" cards both link to dual-diagnosis, which is neither an OCD nor a sleep page (anchor/target mismatch).

- Card-style links swallow entire cards (H3 + paragraph) into one 15-30-word anchor, diluting keyword signal.
- hreflang missing on `programs/outpatient-rehab.html` (a real indexable page), plus `amenity-map`, `intake-success`, `404`.
- `contact.html:201` – malformed `sms:` query string (`?&body=`).

2. Topical Authority – C+

2.1 Content depth: adequate, not authoritative

Body-only word counts (nav/footer/logo-strip excluded):

Page type	Range	Verdict
Treatments (14)	678-2,036; 12 of 14 sit at 670-930	Mid-depth. Only <code>cbt</code> (1,834) and <code>dbt</code> (2,036) reach authority depth vs. the 1,500-word competitive bar
Programs (9 + hub)	827-1,157; hub = 351	Solid; the hub is the thinnest page in its own silo
Locations (11)	1,095-1,287	Consistent and adequate
Blog (6)	922-2,304	Three commercial-intent posts substantive; three support posts ~1,000
Key standalone	faq 2,371 · families 1,578 · team 1,587 · index 1,290 · mental-health 1,073	Fine
Thin	about 768 · our-story 765 · locations.html 771 · contact 523	Thin

The July audit's "460-615-word treatment pages" finding has been fixed – but fixed to ~800, not to 1,500.

2.2 Duplication: not a problem

6-gram shingle overlap, body-only: treatments median **1.8%** (worst pair 15.1%), programs **2.3%**, locations **12.4%** (shared program-card block + directions template – acceptable for geo pages, but the highest-risk set). These are hand-written pages, not stamped templates. The residual overlap on addiction pages is a verbatim SB 855 insurance-parity paragraph repeated on 5 pages, plus shared CTA/detox-referral boilerplate.

2.3 Topic gaps

Absent entirely (zero pages, zero meaningful mentions):

- Insurance carrier pages – Aetna/Cigna/Anthem/BCBS/UHC appear on 46 pages **as logo images only**, never as content. No `/insurance` hub. (Buildout plan Phase 2, "single highest-ROI category" – 0 of 13 pages built.)
- Cost/pricing page (only the blog post exists)
- `/who-we-treat` audience silo – professionals, executives, LGBTQ+, students, veterans, first responders (0 of 9 planned pages built)
- `/levels-of-care` hub – the phrase appears on 21 pages; no page owns it
- Marijuana/cannabis, gambling, adolescent/teen, DUI/court-mandated content

Mentioned but lacking a dedicated page (content already exists in fragments):

Topic	Evidence	Gap
Detox	~85 mentions across 25 files; referral messaging on every location page	No <code>/programs/detox</code> page – the buildout plan's "single highest-priority new page"
Heroin	8 mentions in opioid/fentanyl/med-mgmt	No page for a top-volume term
Benzodiazepines/Xanax	12 mentions in prescription-drugs	No page despite high detox-risk search demand
MAT / Suboxone / Vivitrol	46 mentions, clinically well covered	No standalone page
EMDR	9 files reference it	No page – structural asymmetry: CBT and DBT each have 1,800-2,000-word pages in the same nav group
PHP vs IOP	Covered inside faq/php/outpatient-rehab	No standalone comparison page
Family therapy	<code>families.html</code> is an audience page	No family-therapy <i>program</i> page
Aftercare / relapse prevention	Inside alumni page	No dedicated page

2.4 Cluster structure

The nav defines a real hub-and-spoke skeleton (About / Programs / Treatments with sub-groups / Locations / Blog), but **the hubs are the weakest pages in their own clusters** (programs hub: 351 words; treatments hub: 672; locations: 771), and there is no hub at all for insurance, cost, levels-of-care, or who-we-treat.

Contextual inbound leaders: verify-insurance 57 · iop 55 · team 54 · php 49 · telehealth 46. Blog posts: 7 / 5 / 4 / 3 / 3 / 1. The blog feeds the money pages but receives no topical reinforcement back – it's a terminal leaf, not a supporting cluster.

2.5 FAQ & AI-search readiness

- **43 FAQPage schema blocks; 239 Question/Answer pairs** – near-complete on money pages (29 Q&As on `faq.html` across 7 categories; 4–8 per treatment/program/location page).
- Missing FAQ schema: `programs/index`, homepage, about, team, contact, and 3 of 6 blog posts.
- Outside FAQ accordions, treatment/location body headings are all declarative – zero question-format H2s – which confines extractable Q&A passages to the accordion.
- FAQ page doesn't cover the questions users actually ask: "do you take [carrier]," "what is Suboxone," "do you treat teens," "do you do EMDR," "how long does X stay in your system."
- `llms.txt` is well-formed, current, and E-E-A-T-dense (license number, medical director, full URL inventory).

2.6 Differentiators vs. liabilities

Genuinely differentiated: real local specificity on location pages (transit directions, neighborhood framing – not city-name swaps); honest "we do not provide detox on site" disqualification copy; full 47-page Spanish mirror; 40 real facility photos; the alum-authored first-person blog post; the amenity-map widget (unique, but 10 crawlable words – zero SEO value as built).

Liabilities: uncited stat tiles – `cbt.html` "60-80% Response Rate" and `dbt.html` "50% Reduction in Self-Harm" with no source on the page; zero original/proprietary data (no outcomes, no GSR-specific cost data); insurance logo strip creating the appearance of carrier coverage with nothing behind it.

2.7 Buildout plan vs. built

Plan item	Status
1.1 /programs/detox ("single highest-priority new page")	Not built
1.2 /programs/outpatient	Built – but missing from nav, ES mirror, hreflang
1.3 /levels-of-care hub	Not built
1.4 Expand thin pages to 1,200+	Partial (lifted to ~800-1,150)
Phase 2: /insurance + 12 carrier pages	0 of 13 built
Phase 3: /who-we-treat + 8 audience pages	0 of 9 built
Phase 4: geo pages	Built (11)
Phase 6: 150-post blog roadmap	6 posts; clusters A/B/D/E at zero

3. E-E-A-T – C+

3.1 Authorship & medical review

Strong: all 14 treatment pages carry a visible "Medically reviewed by Dr. Eric Chaghouri, MD" line linked to the team page, backed by `MedicalWebPage` schema with `reviewedBy` + `lastReviewed`. All 11 location pages have both (reviewed 2026-07-07). All 6 blog posts have bylines, avatars, dates, and read times.

Broken:

- **5 of 6 blog posts have no medical reviewer** – visible or structured. The only reviewed post is the *patient-authored* one; the five clinician-authored posts (including all three insurance/cost posts giving financial-medical guidance) have none. The inverse of the correct pattern.
- **11 of 14 treatment pages show `lastReviewed: 2026-03-01`** – ~17 months stale. All 6 blog posts have `dateModified == datePublished` (no freshness signal).
- **9 program pages show the visible review line with no `reviewedBy` / `lastReviewed` schema** – a visible/structured mismatch. Same for `faq.html` (40 FAQs of clinical content).
- Blog bylines are plain text – no link to `/team`, no `Person @id` in `BlogPosting.author`.
- **One reviewer carries the entire YMYL surface.** No LMFT/LCSW co-reviewers; no reviewer for financial content.
- `docs/clinical-review-notes-2026-07-07.md` is a genuinely good internal QA log – but unpublished (zero external signal), and ~30 claims it flags "confirm with Dr. Chaghouri/compliance" remain unresolved (EMDR delivery, acamprosate, alumni "priority admission," "we never contact your employer," the fentanyl "reduces overdose deaths by roughly half" claim).

3.2 Team credentials

7 staff with real names, real headshots (6 of 7), substantive bios, and `Person` schema. Lived-experience bios (26 years of recovery, named prior employers, specific clinical tooling) are the best Experience signal on the site.

- **No license numbers anywhere** – `team.html` says "available on request and verifiable at `search.dca.ca.gov`" (a generic portal, not a record link). CA license numbers are public record; withholding them blocks independent verification of every clinician. This is the single largest missing trust artifact.
- No `Physician` schema type for an MD Medical Director; no `medicalSpecialty`, `NPI`, `sameAs` (Doximity/Healthgrades/LinkedIn), or `image` on any `Person` node.
- `Person` nodes aren't graph-linked to the `Organization` node (inline `worksFor`, no `employee` property on the org) – two floating graphs.
- Sophia Scharpf: no credential, no bio, generic avatar.
- Roster depth: 1 licensed therapist (LMFT) + 1 pre-licensure AMFT + 1 RADT + 1 MD for a facility advertising PHP + IOP + telehealth + individual + group + med management – thin relative to advertised breadth.

3.3 Structured data

Exists: ["MedicalOrganization", "LocalBusiness"] org node on ~54 pages with complete NAP, geo, hours, **DHCS license #191643AP as an identifier**, DHCS + LegitScript `hasCredential`; 26 MedicalWebPage; 40 FAQPage; 52 BreadcrumbList; 46 Person; MedicalCondition/Therapy/Symptom on condition pages.

Missing (high impact): `AggregateRating` / `Review` – **zero occurrences sitewide** despite "5.0 on Google" displayed on the homepage; `Physician` type; org→Person `employee` linkage; Person `sameAs`; `reviewedBy` on programs/faq/blog; `sameAs` limited to LegitScript + Yelp – no Google Business Profile, Psychology Today, SAMHSA locator, or any social profile.

3.4 Trust signals

Present and strong: DHCS license displayed as a clickable hero badge linking to the certificate scan, with a footer-level link to the DHCS provider directory for independent verification (70× sitewide); 988 + SAMHSA crisis lines in every footer; sitewide medical disclaimer; consistent NAP (phone perfectly consistent across 1,400+ instances); substantial privacy policy, terms, and a well-built HIPAA-aware verify-insurance flow (42 CFR Part 2 FAQ).

Missing / weak:

- **No Joint Commission, CARF, or NAATP** – zero mentions. For an addiction facility this is the most conspicuous absence; DHCS licensure is a legal minimum, not an accreditation.
- **Review wall links open the screenshot image files, not the Google Business Profile.** No path to verify a single review; no "see all reviews" CTA; no GBP URL anywhere on the site; no review count behind "5.0 on Google." (Alt text is excellent – full transcriptions – so the content is crawlable, just unverifiable.)
- **"100+ Recoveries" in the H1** – unsourced, undefined, and inconsistent with `our-story.html` ("Clients served"). Highest-risk string on the site under FTC/SB 1228 treatment-advertising scrutiny.
- `foundingDate: 2026` + "100+ clients" + 5.0 rating with nothing anchoring it externally reads as a thin authority profile.
- No editorial/review-policy page, no corrections policy, no outcomes methodology, no exterior/signage photos.
- LegitScript badge is an unlinked `<span>` on the homepage (linked correctly on about/families).

3.5 Citations

- **Treatment pages cite well:** 1–5 authoritative sources each (NIDA, NIMH, SAMHSA, PMC), attributed in-copy.
- **Zero citations on:** `faq.html` (40 clinical FAQs), `mental-health.php`, `iop`, `telehealth`, both silo indexes, all 11 location pages, `about`, `our-story`.
- **Blog: 3 posts have zero external citations.** The insurance post cites MHPAEA, ACA, SB 855, and the ASAM Criteria **without linking any of them**.
- `programs/telehealth.html` is missing the PMC citation the clinical-review notes say was added – likely reverted; verify.
- No semantic references section anywhere – citations are unstyled inline text.

3.6 Experience signals

Strong: lived-experience staff bios; consented, de-identified first-person alum post (HIPAA-handled correctly); 40 real facility photos across dedicated experiential pages; first-hand local detail on location pages; honest detox disclosure.

Weak: only 2 blockquotes sitewide and no testimonial component of any kind; no video; no named founder; the only patient voice is 3 screenshots + 1 post.

4. Prioritized Action Plan

Ranked by (risk reduction + ranking impact) ÷ effort:

#	Action	Fixes	Effort
1	Rewrite the H1 claim – either define and source "100+ Recoveries" or align with "100+ clients served"; reconcile with our-story.html	Critical regulatory/trust risk	XS
2	Link the review wall to the Google Business Profile + add review count + "see all reviews" CTA; add GBP (and Psychology Today/SAMHSA locator) to sameAs sitewide	Unverifiable social proof	XS
3	Publish staff license numbers on team.html (deep-link DCA verification where possible); add Physician type + org↔Person schema linkage	Largest missing trust artifact	S
4	Add contextual homepage links – turn the "why us" and facility card grids into links to treatments/programs/locations hubs; add a featured-post module	Homepage passes zero equity	S
5	Add medical reviewer + fresh lastReviewed to the 5 unreviewed blog posts ; refresh the 11 treatment pages stuck at 2026-03-01; add reviewedBy schema to the 9 program pages + faq	Reviewer gaps, stale dates, visible/structured mismatch	S
6	Build /programs/detox (referral messaging already exists on 25 pages) and /treatments/heroin + /treatments/benzodiazepines (content exists inside opioid + prescription-drugs)	Highest-volume topic gaps, near-zero clinical risk	M
7	Build the /insurance hub + top 5-6 carrier pages – the logos are already on 46 pages; the buildout plan already specifies outlines	Highest-ROI missing axis	M-L
8	Wire service → blog links (iop → first-week post, cbt/dbt → comparison post, verify-insurance → all three money posts) and add outpatient-rehab to the nav + ES mirror + hreflang	One-way blog, orphaned head-term page	S
9	Add citations to faq, mental-health, php/iop/telehealth, and the 3 uncited blog posts; link MHPAEA/SB 855/ASAM in the insurance post; source or remove the CBT/DBT stat tiles	Uncited clinical/financial claims	S-M
10	Fix cross-silo holes – treatment pages link to /locations ; each city page links 2-3 locally relevant treatments; fix the cbt.html OCD/insomnia anchor mismatches; balance the neighbor-city link rotation	Silo isolation	M
11	Pursue accreditation (Joint Commission or CARF; join NAATP meanwhile) and publish an editorial/review-policy page based on the existing internal clinical-review process	Missing authority anchors	L (business)
12	Longer-term: /levels-of-care hub + PHP-vs-IOP comparison page, EMDR page, expand the 12 mid-depth treatment pages toward 1,200-1,500 words, resume the blog roadmap (clusters A/B: "how long does X stay in your system," signs/withdrawal)	Depth + coverage	L

Sources: three parallel full-site analyses (link graph, content inventory, E-E-A-T signals) of the repository HTML as of commit 58805b3, cross-referenced against golden-state-rehab-seo-buildout-plan.md, docs/seo-audit-report.md, docs/seo-action-plan.md, and docs/clinical-review-notes-2026-07-07.md.